

Spina bifida support

Regenerative biologic adjunct · intramuscular administration

An adjunctive framework intended to support neural signaling, reduce neuroinflammation, and support systemic repair mechanisms alongside standard medical and surgical management. This therapy is not intended to cure spina bifida and is investigational and off-label.

Inclusion

- Diagnosed spina bifida (myelomeningocele, meningocele, or occulta)
- Pediatric or adult patients per clinic policy
- Medically stable without active infection
- Informed consent obtained

Exclusion

- Active systemic infection
- Active malignancy
- Pregnancy unless approved by the Medical Director

Dosing & administration

REGIMEN A · TOTAL

5 cc

EXM 2 cc + PV-MAX 3 cc, IM

NEEDLE / OBSERVATION

22–25 G

observe 15–30 min post-procedure

REGIMEN B · NOVA PRODUCT DISTRIBUTION (7 cc TOTAL)

- NOVA-MSC FLEX: 2 cc per glute, bilaterally
- NOVA-CORE: 1.5 cc per glute, bilaterally
- Preferred site: gluteal musculature (volume tolerance, systemic absorption)
- Alternative: deltoid, maximum 2 cc per deltoid
- Aspirate prior to injection and inject slowly
- Regimens A and B are alternatives, not additive — confirm which applies before dosing.

OUTCOME MEASURES

NEUROLOGIC STABILITY

PAIN / SPASTICITY

FUNCTIONAL MOBILITY

QUALITY OF LIFE

PREPARATION & HANDLING

- Thaw according to manufacturer specifications
- Glycerol-based products do not require washing
- Use within 15–30 minutes post-thaw
- Maintain strict sterile technique throughout

TREATMENT SCHEDULE

Single initial administration. Repeat dosing may be considered every **4–8 weeks** based on clinical response; re-evaluate after **2–3 sessions**.

ADJUNCTIVE CARE

Continue standard medical and surgical management. Consider nutritional optimization (B-complex, magnesium, omega-3) and physical or neuro-rehabilitation as indicated.

RISKS & SAFETY

Injection-site discomfort, bruising, or transient inflammatory response; potential site infection. No known tumorigenic risk has been identified based on the acellular nature of the biologics.

OVERSIGHT

All treatments must be ordered and supervised by a licensed physician experienced in regenerative medicine, and must comply with applicable state and federal regulations, institutional policy, and informed-consent requirements. No claims of disease cure are made.

Today you received intramuscular injections of cell and exosome products into the hip and buttock muscles. This note is written for the patient and family. The products used are made from donated birth tissue and are not FDA-approved treatments. This page tells you what is normal in the days ahead, how to care for yourself, and when to call us.

DATE OF TREATMENT

SITE(S) TREATED

NEXT VISIT

WHAT TO EXPECT

Changes are gradual — do not judge the result in the first days

FIRST 48 HOURS

Soreness at the injection sites is common, and a low-grade temperature for under 24 hours can occur. Normal activity as tolerated is fine.

FIRST WEEKS

Physical therapy, bracing, and bladder and bowel routines continue exactly as before — this treatment adds to that care, never replaces it.

BY FOLLOW-UP

Bring your notes and diary. Progress is measured against PT goals and the baseline exam, not day-to-day impressions.

DO

- Keep every physical therapy appointment
- Bathe normally after 24 hours
- Use acetaminophen (Tylenol), dosed by weight, for soreness
- Keep the bladder and bowel routine unchanged

AVOID

- Anti-inflammatory pain relievers for 72 hours unless your physician says otherwise
- Hot soaking baths for 24 hours
- Massage of the injection sites for 48 hours
- Judging results in the first days

CALL US RIGHT AWAY IF

For an emergency, call 911 first

- Fever above 101 °F
- Unusual sleepiness or lethargy
- Spreading redness, warmth, or swelling at an injection site
- New weakness, or any change in bladder or bowel function

CLINIC PHONE

AFTER-HOURS PHONE

NOTES FROM YOUR VISIT